

Customer Discovery Synthesis and Business Model Canvas

STUDENT	DATE	VENTURE	INTERVIEWS
Fanny Polanco	June 2026	AI-Powered Pre-Diabetic Food System	5 completed

1. Interview Insights Report

I conducted 5 interviews with adults managing pre-diabetes or blood sugar risk. Three interviewees (Tere, Grace, Mary) are confirmed pre-diabetic, working full time, and managing family caregiving on a tight grocery budget. One (Nelsy) has high blood pressure, not pre-diabetes — her data validates the structural failure pattern but not the blood sugar logic. One (Veronica) has no blood sugar framework and dislikes vegetables — she is excluded from the primary target profile. Interviews were conducted in person and over the phone. Notes and transcripts are submitted separately.

a. Pattern Map

Confirmed Patterns (3+ interviewees independently)

Weekly produce waste is universal and accepted as inevitable. All 5 of 5 interviewees confirmed weekly produce waste — not as a problem they are trying to solve, but as a recurring fact of life. Tere: *"I buy vegetables with good intentions, but by the time I get around to cooking, they're already soft or moldy. I feel guilty throwing them away, but it happens every week."* Grace: *"They sit in the fridge until they're gross."* Mary: *"I buy them because I know I should eat healthier, but I never plan meals around them."*

Grocery shopping happens without a list or meal plan. 5 of 5 interviewees considered buying healthier food and did not, because they could not connect the item to a specific meal. Tere: *"I didn't have a list. I just walked around grabbing things I usually buy."* Veronica: *"I bought things that 'seemed healthy,' but I didn't know what meals to make."*

Inspiration does not convert to action — across every platform and source. 5 of 5 interviewees use TikTok, YouTube, Pinterest, Google, or a trusted person for food help. Not one converts to a meal consistently. Mary: *"Those recipes look good and then I never make them."* Nelsy: *"I call mom and she gives me these recipes. They are healthy, but I don't end up cooking them."*

Every structured resource was tried and abandoned. 5 of 5 interviewees tried at least one tool and stopped: a cookbook (Mary), a diabetes app (Grace), a meal kit (Nelsy), a free app (Veronica), and YouTube/TikTok recipes (Tere). Each failed for a different reason — complexity, irrelevance, price, friction, missing ingredients — which means no single improvement to an existing tool solves the problem.

Speed and ease override health when there is no plan. 5 of 5 interviewees default to the fastest option when the plan fails. Nelsy: *"I decide based on what's fast."* Veronica: *"I just went with whatever was easiest."*

Overspending despite not buying healthier food. 3 of 5 interviewees (Mary, Veronica, Tere) spent more than intended without filling the cart with healthy items. Without a plan, the budget is wasted on defaults, not improved choices.

Contradictions

They know a specific food is bad for them and eat it every day. Tere knows rice worsens her blood sugar and eats it every night. Grace knows ramen is bad and eats it for lunch. Knowledge is present. Behavior does not change. The product's answer must be that the system removes the decision entirely — but this depends on the customer following the plan in advance, which is the behavior the venture is trying to produce. The circularity is real and unresolved.

Trusted, personalized human guidance does not produce action. Nelsy's mother gives her healthy, personalized recipes from a completely trusted source. She does not cook them. This reframes the AI trust question: the right question is not whether the customer will trust an AI — it is whether the output is complete enough that she does not have to do anything after receiving it except follow it.

A free product was abandoned for reasons unrelated to price. Veronica tried a free app and abandoned it because it felt like homework. Removing price entirely does not solve adoption. Product design — low friction, pull-based, complete output — is an equal or greater barrier.

Surprises

Produce waste carries guilt, not just financial frustration. Tere's language was emotional: *"I feel guilty throwing them away."* The original framing positioned waste as a financial failure. "Stop feeling guilty every week" and "save money on groceries" are different value propositions aimed at different motivators. The guilt framing may be more resonant.

No interviewee mentioned a doctor or healthcare provider as a food resource. All five are pre-diabetic or high risk. Not one named a doctor, nurse, or dietitian referral as a source of food guidance. Sources named: TikTok, YouTube, Pinterest, Google, a cookbook, a diabetes app, a meal

kit, a mother. The HCP referral channel has zero direct support from these interviews.

Grace is already trying to solve the problem — and her solution created a new problem.

Grace batch-cooks all vegetables at once and found they become soggy by day three. The product must account for palatability across multiple days, not just planning.

b. Key Quotes

#	Interviewee	Quote	Why It Matters
1	Tere (Int. 1)	"I have the intention, but not a good system."	The clearest confirmation of the venture's core premise. The customer is not unmotivated — she is unsupported. Names the exact gap the product is built to fill, in her words, without prompting.
2	Tere (Int. 1)	"I know the rice isn't great for my sugar, but I was too tired to think of something else."	Challenges the assumption that behavior change follows from knowledge. The product needs to remove the decision point entirely, not just inform it.
3	Tere (Int. 1)	"I feel guilty throwing them away, but it happens every week."	Produce waste is emotional, not just financial. Guilt may be the more resonant value proposition hook for this customer.
4	Mary (Int. 2) △	"I had oatmeal for breakfast because it's 'healthy,' even though I'm not sure about the sugar."	Directly contradicts the original claim that the problem is not nutritional knowledge. The knowledge gap and system gap coexist.
5	Nelsy (Int. 4) △	"I call mom and she gives me these recipes. They are healthy, but I don't end up cooking them."	The most important challenge in the data set. A trusted, personalized human source does not convert to action. The product's job is completeness of output, not credibility of source.
6	Grace (Int. 3) △	"I tried a diabetes app once, but it felt like it was made for older people. Not helpful."	Lost Grace on cultural fit, not features. Design, tone, and demographic match are adoption factors — not secondary polish.

7	Veronica (Int. 5) △	"I tried a free meal prep app, but it felt like homework. Too many notifications."	Abandoned at zero cost. Price is not the only barrier. A product requiring ongoing engagement imposes a friction cost that overrides free.
8	Veronica (Int. 5) △	"I don't like vegetables. I know I need to eat them though."	Defines the boundary of the addressable customer. Veronica is excluded from the primary profile — her barrier is preference, not logistics.

△ = Quote challenges the venture idea

c. Assumption Scorecard

Assumption	Evidence For	Evidence Against	Verdict	What Changes
The market is saturated with pre-diabetic tools	PlateJoy raised \$12.3M and built a CDC-recognized diabetes prevention program. Klinio and eMeals exist.	Zero active users of a pre-diabetic product across five interviews. Grace tried a diabetes app and found it irrelevant. No direct-to-consumer product built for this customer exists.	Disproven	The direct-to-consumer space is largely empty. Absence of competition signals that serving this customer well is genuinely hard. Stop treating saturation as the primary risk.
Pricing is a secondary differentiator	None. No interviewee indicated price was not a concern.	Nelsy canceled a meal kit on price. Grace operates on a \$60 hard grocery ceiling.	Disproven	Price is a gate, not a differentiator. It must be a primary design constraint from day one. Who actually pays is an unresolved and real problem, not a detail

Competitive analysis: \$5–15/month is a structural barrier. PlateJoy pivoted to B2B partly because it could not charge the consumer directly.

to decide later.

Target customer understands basic blood sugar logic

Tere: "I know the rice isn't great for my sugar." Grace: "I know ramen is bad for sugar." Mary tried a low-carb cookbook, indicating directional awareness.

Mary: "I'm not sure about the sugar" in oatmeal. Grace bought bread and rice on the same trip as ramen with no flag. Veronica shops by "seemed healthy" with no blood sugar framework.

Partially Confirmed

Retire the claim "the problem is not a lack of nutritional knowledge." Knowledge is surface-level and inconsistent. Brief rationale must accompany recommendations so customers can spot-check them.

The core problem is a system breakdown, not a knowledge gap

Tere: "I have the intention, but not a good system." All five confirmed produce waste, unplanned shopping, and tool abandonment.

Mary does not know whether oatmeal is safe — that is a knowledge gap. Veronica has no blood sugar framework at all. Grace believes walking compensates

Partially Confirmed

System gap is primary and confirmed. Knowledge gap is secondary and real — not absent. Design for the customer who has partial knowledge and will spot-check outputs.

for ramen.

Pre-diabetic adults would trust an AI system for food planning	Tere wants guidance but "not something time-consuming." Mary wants something simpler than a cookbook. Grace wants a "trusted, filtered, personal recommendation."	Grace: diabetes app "felt like it was made for older people." Nelsy trusts her mother over apps. Veronica abandoned a free app. Every tool tried was abandoned. No interviewee was asked about AI directly.	Inconclusive	This assumption was never tested. Must be tested before significant product investment. Suggested question: "If something built your weekly plan around your actual budget, schedule, and blood sugar — and got it right — would it matter whether a person or an AI built it?"
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2. Venture Direction Decision

Refined Problem Statement: Pre-diabetic adults managing work and family caregiving have enough awareness that certain foods worsen their condition but no practical system to act on that awareness consistently. Every week they buy produce with good intentions that spoils, shop without a list, overspend, and default to meals the family will accept rather than meals safe for their blood sugar. The barrier is not motivation — every interviewee demonstrated intent. It is the absence of a system that connects grocery budget, weekly schedule, family food preferences, and blood sugar logic into something repeatable that does not require starting from scratch each week.

What changed from Week 1: The original problem statement claimed "the problem is not a lack of nutritional knowledge." The interviews do not support that claim. Mary did not know whether oatmeal was safe for her blood sugar. Grace bought bread and rice on the same trip as ramen without recognizing the same glycemic problem. A knowledge gap is real and coexists with the system gap. The refined statement acknowledges both without making the product responsible for solving both.

Refined Target Customer: A pre-diabetic adult, 35 or older, diagnosed by a doctor, who works full time and is a primary caregiver for a family. They operate on a grocery budget tight enough that \$60 is a ceiling, not a preference. They know one or two foods that affect blood sugar but cannot

apply that logic consistently. They have tried at least one structured resource and stopped using it. My beachhead is Profile 1: Tere, Grace, and Mary. Nelsy (high blood pressure, not pre-diabetic) and Veronica (no blood sugar framework, dislikes vegetables) are excluded from the primary profile.

Chosen Direction: Remix 1 — direct-to-consumer AI-powered weekly food system. Takes a family's budget, refrigerator inventory, local grocery sales, and weekly schedule and generates a fresh, blood-sugar-safe meal plan every week. Two core leverage points: (1) Ingredient-first planning: plan three meals around one bag of green beans before buying it, breaking the waste loop at the source. (2) Built-in variety engine: same ingredient, different preparation each day, breaking the fatigue loop. B2B institutional (Remix 2) and grocery chain partnership (Remix 3) remain viable as future layers once the core product is validated.

What changed from Week 1: In Week 1, I believed pricing was secondary and that the market was saturated. Both assumptions were disproven. The target customer narrowed from all pre-diabetics to Profile 1 only — those with partial blood sugar awareness who have tried and abandoned tools. The problem statement walked back the knowledge gap claim after Mary's interview.

Biggest remaining risk: AI trust for a medically sensitive recommendation — completely untested. No interviewee was asked about AI directly. The entire product depends on a customer who has already abandoned every digital tool she tried accepting a plan built by an AI for a diagnosed medical condition. Evidence needed in the next two weeks: a direct answer from Tere, Grace, and Mary to the question: "If something built your weekly plan around your actual budget, schedule, and blood sugar — and got it right — would it matter whether a person or an AI built it?"

3. Business Model Canvas

Each block includes current best answer and evidence note. Blocks with weak or no evidence are flagged in red — these are validation priorities for the next two weeks.

Customer
Segments

Primary: Pre-diabetic adult, 35+, working full time, primary caregiver, ~\$60 grocery ceiling, partial blood sugar awareness, has tried and abandoned at least one structured tool. Beachhead: Tere, Grace, Mary.

Evidence: 3 of 5 interviewees confirmed the pain. Payer identity unresolved — if this customer cannot pay, she is the user, not the customer, and the segment definition changes.

For the pre-diabetic family manager: Eliminates weekly produce waste. Converts inspiration to execution. Removes the weekly planning restart. Handles blood sugar logic without requiring complete nutritional knowledge. Works for the whole household, not just the person managing blood sugar.

Value Propositions

Evidence: All 5 value statements trace to specific interview quotes. Tere: "I have the intention, but not a good system." AI trust not yet validated — value cannot be delivered if the customer will not follow a plan built by an AI.

Channels

Awareness: TikTok and YouTube — 3 of 5 interviewees use these for food help. Content that intercepts the execution gap (she finds a recipe and can't make it) meets her where she already is.

Delivery: SMS or WhatsApp — no download required, already open daily.

Strongest structural option: Grocery chain app (Remix 3) — already on her phone, opened weekly for a directly related task, solves payer and channel simultaneously.

Evidence: TikTok/YouTube supported by 3 interviews. SMS delivery untested. HCP referral has zero interview support — inference only.

Customer Relationships

Pull-based, not push-based — the customer comes when she needs it, not when the app sends a notification. **Complete output, not guidance** — she receives a ready-to-execute plan and grocery list, not advice she has to convert. **Trusted, not clinical** — the tone matches her life, not a medical protocol.

Evidence: Veronica: "It felt like homework. Too many notifications." Nelsy: mom's recipes not cooked despite trust. Grace: app felt made for older people. AI trust question lives here — untested.

Revenue Streams
WEAK

Option 1: Direct subscription (<\$5/month hypothesis — unconfirmed)

Option 2: Institutional B2B — clinic or employer pays (\$30–50/member/month; strong ROI case for payer, 6–12 month sales cycle)

Option 3: Grocery chain license fee — chain funds free access for shoppers in exchange for basket size lift

No interviewee was asked what they would pay. Willingness to pay is completely untested. No price point confirmed. Revenue model cannot be chosen until this is tested directly.

Key Resources

Validated glycemic logic library — the curated, clinically validated set of blood sugar rules that power recommendations. Primary defensible asset. Hardest for a competitor to replicate quickly.

AI inference capability — prerequisite for the "why now" argument.

Grocery price data — required for the budget constraint to be honored.

Founder's lived experience — direct access, domain credibility, and lived understanding of the failure modes the product solves.

Clinical advisor relationship — required before the product touches any user with a diagnosed condition.

Key Activities

Core (all paths): Weekly plan generation; glycemic logic curation and validation; grocery price data integration; low-friction onboarding; clinical accuracy QA.

Go-to-market (path-dependent): Short-form content creation on TikTok/YouTube (direct-to-consumer path); B2B partnership development (institutional or grocery path).

Immediate priority (all paths): Willingness-to-pay testing — no interviewee was asked what they would pay.

Key Partnerships
WEAK

Required in any version: Clinical advisor (RD or CDE) — not optional before the product reaches anyone managing a diagnosed condition; AI inference provider; grocery price data source.

Contingent on revenue model: Regional grocery chain (Remix 3) or community health clinic / FQHC / employer (Remix 2).

No partnership is confirmed. Clinical advisor unidentified. Grocery data source unchosen. All three required partners are currently unengaged.

Cost Structure
WEAK

Material cost categories: AI inference (variable, scales with usage); clinical advisor and glycemic logic validation; customer acquisition (cost varies by channel); legal and liability exposure from incorrect glycemic guidance.

No figures available. Cost structure follows revenue model — which is unresolved. Liability exposure from incorrect glycemic guidance is unscoped and non-optional. Clinical validation is uncoded. Both must be understood before the product is shown to users with a real diagnosis.

Validation priorities for the next two weeks: (1) Ask Tere, Grace, and Mary about AI trust and willingness to pay — these two questions unlock eight of nine canvas blocks. (2) Identify and engage a clinical advisor — required before any product touches a user with a diagnosed condition. (3) Resolve the payer question — direct consumer, institutional, or grocery chain — before building further.

4. Claude Code Session Log

Session log exported from Claude Code. Submitted as a separate file (session-log.md). The log shows the full working session: interview transcripts fed into Claude Code, synthesis creation, pattern analysis, pushback on AI output, assumption scorecard, venture direction decision, and Business Model Canvas built block by block.

